Tracheostomy Tube Emergencies

AR-10 | Airway & Respiratory

Quick flow

01 Tracheostomy or Laryngectomy? [EMT / AEMT / Paramedic]

Ask caregiver and inspect stoma/tube. If uncertain, apply oxygen to both face and stoma.

02 Remove Simple Obstruction [AEMT / Paramedic]

Remove speaking valve, cap, HME/filter, and inner cannula if present. Do not remove voice prosthesis.

03 Suction + Reassess [AEMT / Paramedic]

Use patient-specific catheter/depth when known. Never force resistance.

04 Catheter Will Not Pass / Tube Displaced? [AEMT / Paramedic]

Deflate cuff if present, reassess face/stoma airflow, and remove the tracheostomy tube when obstruction/displacement is established.

05 Ventilate Through Available Airway [EMT / AEMT / Paramedic]

Trach: face and stoma as appropriate. Laryngectomy: stoma only with pediatric mask/stoma interface.

06 Confirm + Transport [EMT / AEMT / Paramedic]

Waveform $EtCO_2$, SpO_2, secure any rescue tube, bring spare equipment/caregiver, and notify destination.

Overview

- First determine whether the patient has a tracheostomy or total laryngectomy; use caregiver knowledge, emergency information, and the patient's equipment.
- Prioritize oxygenation, remove simple obstruction, suction, and recognize that a blocked or displaced tracheostomy tube may need prompt removal rather than repeated ventilation attempts.

Provider-Level Actions

Use the least invasive effective airway intervention and escalate only when oxygenation or ventilation remains inadequate.

EMT

- Use caregiver and patient equipment, determine tracheostomy versus laryngectomy, provide oxygen, assist ventilation at the stoma, and prepare suction/replacement equipment.
- If uncertain whether a patient has a laryngectomy, apply oxygen to both face and stoma until anatomy is confirmed.
- A confirmed total laryngectomy disconnects the upper airway from the lungs; oxygenate and ventilate only through the stoma.

AEMT

- Perform all EMT care plus remove obstructing valves/caps/HME/inner cannula, perform authorized suctioning, and assist with authorized tracheostomy replacement.
- Do not make repeated attempts to ventilate through a potentially displaced or obstructed tracheostomy tube. An endotracheal tube through the stoma is not an AEMT procedure in this protocol.

Paramedic

- Perform all prior care and manage persistent obstruction, displacement, pneumothorax, respiratory failure, or rescue ventilation with advanced airway support.
- For a mature tract when replacement is appropriate, use the patient’s spare tube or one size smaller; an appropriately sized cuffed ETT through the stoma is a paramedic rescue option when necessary.

- For a fresh/immature tract, do not blindly replace the tube because of false-passage risk; prioritize oxygenation/ventilation and rapid transport.

Indications

- Tracheostomy or laryngectomy patient with distress, cyanosis, altered mental status, copious secretions, obstruction, displacement, decannulation, or bleeding.

Treatment steps

- First determine tracheostomy versus total laryngectomy. If uncertain, apply oxygen to both face and stoma. For confirmed laryngectomy, the stoma is the only airway; do not attempt oral/nasal ventilation or intubation.
- Do not remove a tracheoesophageal voice prosthesis during an emergency.
- For suspected tracheostomy obstruction, immediately remove external attachments: speaking valve, cap, HME/filter, and inner cannula if present.
- Then attempt suction with the patient's usual catheter/depth when known. If unknown, use the smallest appropriate catheter and stop at resistance; do not use a fixed depth.
- If the suction catheter will not pass, treat the tube as obstructed or displaced: deflate the cuff when present, reassess face and stoma airflow, and remove the tracheostomy tube rather than repeatedly attempting to ventilate through it.
- After tracheostomy tube removal, provide oxygen to face and stoma. Use oral/nasal BVM if the upper airway is patent; occlude the stoma if necessary to obtain chest rise.
- For laryngectomy rescue ventilation, use a pediatric mask or appropriate stoma interface over the stoma; confirm chest rise and waveform $EtCO_2$.
- After any stoma tube/ETT placement, confirm with waveform $EtCO_2$, chest examination, SpO_2, depth, and securement.
- Significant tracheostomy bleeding is potentially life-threatening: provide oxygen/ventilation, activate rapid transport, and contact Medical Control early.

Warnings / precautions

- Do not force a suction catheter; inability to pass suggests obstruction or displacement.
- Deflate the cuff before removing a cuffed tracheostomy tube.
- Do not attempt oral or nasal ventilation after total laryngectomy; ventilate through the stoma.
- Do not remove a tracheoesophageal voice prosthesis during an emergency.
- Significant tracheostomy bleeding is potentially life-threatening; expedite transport and contact Medical Control.

Clinical pearls

- If airway anatomy is uncertain, oxygen to both the face and stoma is safest until laryngectomy is excluded.
- A blocked or displaced tracheostomy tube should be removed once established, rather than treated as a route for repeated ventilation attempts.

Related pathways / contacts

AR-01 Adult Airway - Open for progressive respiratory failure or rescue airway support.

AR-08 Post-Intubation / BIAD Management - Open after a rescue airway is placed for confirmation, monitoring, and transport care.

AR-09 Ventilator Emergencies - Open for ventilator alarm, equipment failure, or home-ventilator troubleshooting.

Source and references

Claiborne County EMS approved protocol manual. | Claiborne County EMS formulary, linked protocol pathways, and agency release guidance.

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